

[SYNTHETIC SAMPLE] Fictional record generated for the DELPHi prototype demo. Names, institutions, and statements are invented; the layout imitates a generic field-medical format. Not a real document and not derived from any real record.

[CONFERENCE REFERENCE] The meeting named above is a real, publicly announced scientific congress and is used here only as the setting of this fictional record. No session content, abstract, poster, or presented data from that meeting is reproduced or paraphrased; every person and conversation below is invented.

Congress Attendance Report — American Epilepsy Society Annual Meeting (AES) 2025

Meeting period: December 2025 | Author: Colin Ferreira

Scope: unsolicited conversations held on-site. Session content and presented data are deliberately not reproduced here.

Conversation — Gaspar Ostrander, MD (Riverstone Comprehensive Epilepsy Clinic)

Introduced by colleague from same region and talked for few minutes. Brought up, unprompted, several Lennox-Gastaut patients on four or more medications with unchanged seizure counts. The practice lost two medical assistants and has not backfilled, so referral queue keeps slipping.

Conversation — Yesenia Wainwright, MD (Beacon Hill Neurology Associates)

A short exchange over the coffee queue. Described preference for slower titration in general, unrelated to any particular patient. Compared notes on which sessions were worth the time. Described how well it works in focal epilepsy and how little that helps the generalized patients in this panel. Requested any published material covering generalized seizure types.

Conversation — Saoirse Inverness, MD (Granite Bay Epilepsy Center)

A short exchange over the coffee queue. Monitoring intervals during titration are set by clinic routine here rather than by protocol. Noted the gap for Lennox-Gastaut patients, described as the most difficult conversation in this clinic.

Conversation — Rosalind Kettleborn, MD (Ember Lake Medical Group)

Stopped to talk on way out of a session block, with no agenda on either side. Talked through pediatric-to-adult transfers in general terms, without any specific case. Flagged the Lennox-Gastaut patients whose drop attacks continue on every combination tried.

Conversation — Rosalind Wainwright, MD (Sable Creek Medical Group)

A short exchange over the coffee queue. Referenced a session summary from a colleague in passing. Titration scheduling is handled by nursing staff, which practice finds workable. Would like whatever literature exists on generalized tonic-clonic populations. Framed it as patients with generalized tonic-clonic seizures who cycle through the same medications with the same result.

Conversation — Imani Grierson, MD (Foxglove Health Neurology)

Stopped to talk on the way out of a session block, with no agenda on either side. The patient portal is still being sorted out, and that took up the first part of the convo. Referral patterns into this clinic have

changed, and surgical workup list is bigger share than it was. Said the hardest population in this practice is how families of Lennox-Gastaut patients keep asking what else exists.

Conversation — Anouk Oyelaran, MD (Marbury Neurological Care Center)

We spoke standing near exhibit aisles, briefly. Keeps seeing post-stroke epilepsy patients, now a visible share of new referrals. Mostly small talk about the program and the venue.

Conversation — Tobias Rusnak, MD (Sable Creek Medical Group)

Stopped to talk on way out of a session block, with no agenda on either side. Noted the interaction checks required before starting anyone elderly, which slow the decision down. Benefit verification is the step that most often delays a start here. Referral patterns into this clinic have changed, and refractory group is bigger share than it was.

Conversation — Felicity Ashgrove, MD (Foxglove Health Neurology)

Caught between sessions in the poster hall; the convo was short and entirely unprompted. Closed there — nothing actionable this visit. Described post-stroke epilepsy, which this clinic sees more of every year.

Conversation — Gemma Steinmetz, MD (Vantage Park Regional Epilepsy Program)

A short exchange over the coffee queue. Coverage paperwork is handled by one person here, which came up as a general frustration. The travel schedule this year came up, and little else. Brought up, unprompted, the gap for Lennox-Gastaut patients, described as the most difficult conversation in this clinic.

Conversation — Eloise Thackeray, MD (Alder Point Neurology Associates)

The convo happened while waiting for session room to clear. The travel schedule this year came up, and little else. Closed there — nothing actionable this visit. Asked for the drug-drug interaction table for geriatric polypharmacy cases.

Follow-ups have been logged in the interaction system.